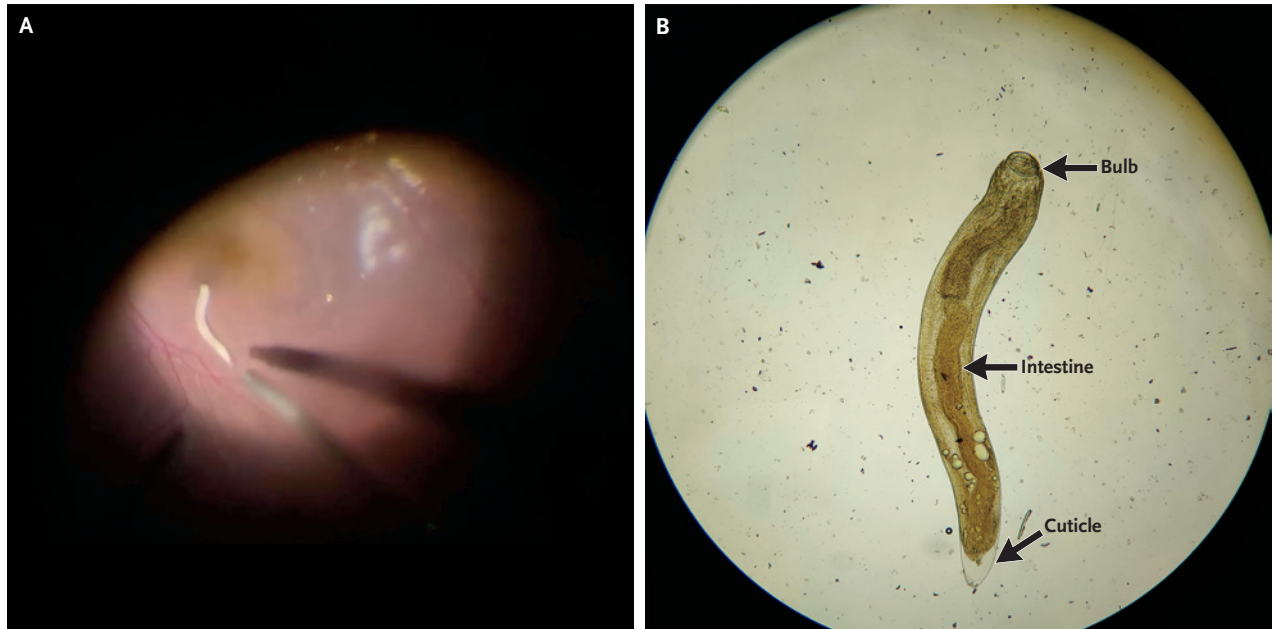


IMAGES IN CLINICAL MEDICINE

Ocular Gnathostomiasis



A 35-YEAR-OLD MAN PRESENTED TO THE OPHTHALMOLOGY CLINIC WITH an 8-month history of redness and blurry vision in the left eye. On physical examination, the left eye was found to have conjunctival injection and a fixed, dilated pupil. Visual acuity in the left eye was 20/80, and slit-lamp examination showed panuveitis. On funduscopy, a worm was seen moving sluggishly in the posterior segment. Examination of the skin and the rest of the neurologic system was normal. A pars plana vitrectomy was performed to remove the worm (Panel A and video). Under light microscopy, a larval-stage nematode with a cephalic bulb, thick cuticle, and well-developed intestine was identified (Panel B); these features were consistent with *Gnathostoma spinigerum*. Computed tomography of the brain, orbits, chest, abdomen, and pelvis was unremarkable. A diagnosis of ocular gnathostomiasis was made. Gnathostomiasis is acquired through ingestion of undercooked freshwater fish, poultry, snake, or frog in regions where the disease is endemic — the first two of which this patient from rural central India had consumed. Treatment with oral and ocular glucocorticoids was given because of the severity of the patient's uveitis. A course of albendazole was also prescribed. At the 8-week follow-up, his symptoms had resolved but his visual acuity remained 20/40 in the left eye owing to the development of a cataract.

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A video is
available at
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